

APPLICATION OF MEDICAL-LEGAL ETHICS

This unit standard specifies the competencies required to apply medical legal ethics in POTT. It involves acquiring knowledge on the medical-legal and ethical issues, applying medical-legal ethics, and liaising with law enforcement team. It also involves documenting medical legal ethical issues in POTT.

Summary of Learning Outcomes

- i. Acquire knowledge medical- legal ethics in POTT
- ii. Apply medical legal ethics in POTT
- iii. Liaise with law enforcement team on medical legal and ethical issues
- iv. Document medical legal- ethical issues in POTT

Learning Outcome 1: Acquire knowledge medical- legal ethics in POTT

Introduction to the learning outcome

This learning outcome specifies the roles and responsibilities of PTT staff as per WHO, relevant regulations/laws and workplace policy. It includes ethics deals with how providers apply a moral code of conduct to patients in a healthcare setting, taking into account the patients' self respect, individuality, safety and welfare.

Performance Standard

- i. Roles and responsibilities of PTT staff are identified as per WHO, relevant regulations/laws and workplace policy.
- ii. Medical- legal and ethical issues in PT are identified as per the WHO and relevant sources of law.
- iii. Medical legal guidelines are acquired in accordance with workplace policy and relevant laws.

Information Sheet

Definition of terms

applied ethics - the practical application of moral standards to the conduct of individuals involved in organizations

bioethicists - persons who specialize in the field of bioethics

bioethics - also called biomedical ethics, the moral dilemmas and issues of advanced medicine and medical research

comparable worth - also known as pay equity, the theory that extends equal pay requirements to all persons doing equal work

compassion - ability to have a gentle, caring attitude toward patients and fellow employees

cost/benefit analysis - also called utilitarianism, an ethical approach in which the benefit of the decision should outweigh the costs

empathy - the ability to understand the feelings of another person without actually experiencing the pain or distress that person is going through

ethics - the branch of philosophy relating to morals and moral principles

integrity - the unwavering adherence to one's principles; dedication to maintaining high standards

laws - rules or actions prescribed by a governmental authority that have a binding legal force

medical ethics - moral conduct based on principles regulating the behavior of healthcare professionals

medical etiquette - standards of professional behavior that physicians use for conduct with other physicians

medical practice acts - laws established in all 50 states that define the practice of medicine as well as requirements and methods for licensure in a particular state.

Morality - the quality of being virtuous or practicing the right conduct

principles of autonomy - right to make decisions about one's own life

principle of beneficence - in doing good, we must not harm patients while we are trying to help them. Principle of justice warns us that equals must be treated equally

principle of non-maleficance - means "first do not harm"

quality assurance - gathering and evaluating information about the services provided as well as the results achieved and comparing this information with an accepted standard

sanctity of life - sacredness of human life; all human beings must be protected.

sexual harassment - unwelcome sexual advances or requests for sexual favors

sympathy - pity for someone else

tolerance - respect for those whose opinions, practices, race, religion, or nationality differ from our own

administrative law - a branch of law that covers regulations set by government agencies

assault - imminent apprehension of bodily harm

battery - requires bodily harm and unlawful touching (touching w/o consent of patient)

breach - neglect of an understanding between two parties; failing to perform a legal duty

breach of contract - the failure, without legal excuse, to perform any promise or to carry out any of the terms of an agreement; failure to perform a contractual duty.

case law - also called common law; case law is based on decisions made by judges

civil law - all law that is not criminal, including the law of torts (personal wrongs) and contract, property, maritime, and commercial law.

Competent - capable of making a decision without mental confusion due to drugs, alcohol, or other reasons

constitutional law - the inviolable rights, privileges, or immunities secured and protected for each citizen by the Constitution of the United States or by the constitution of each state

contract law - that division of law that includes enforceable promises and agreements between two or more persons to do or not to do a particular thing.

criminal law - those laws intended to protect society from the wrongdoing of an individual

defendant - person or group of people sued civilly or prosecuted criminally in a court of law

plaintiff - the person or party who brings a case to court

regulations - rules or laws made by agencies

tort law - Law that deals with harm to a person or a person's property.

Confidentiality - professionals involved in the patient's care, or under subpoena, or in a court of law, or when the patient has signed a release of confidentiality

medicare - health care for people over the age of 65

national practitioners data bank (NPDB) - a listing of names that assists with peer review of physicians.

preferred provider organization (PPO) - a managed care concept in which the patient must use a medical provider who is under contract with the insurer for an agreed-upon fee in order to receive co-payment from the insurer.

primary care physician (PCP) - HMO-designated physician to manage and control an enrolled patient's medical care.

Sources of Law

The sources of law are the origin or where specific laws derive their source from. In the world over, laws have been found to derive their source from the constitutions, Judicial decree, religion and relevant legislation i.e. parliamentary acts.



Customs - Customs play an important role in the framing of the laws. Most of the laws that came from customs are recognized by state later. Since ancient period we can notice that the dispute among tribals were resolved by the head of the Tribes using their customs and traditions. Customs became laws when tribes extended into the formation of state. State cannot actually ignore the customs of the country. The common law of England for example sprang mainly from customs. For example Bull-Taming sport culture of Tamils of India resulted into the creation of Jallikattu Law in 2017. (Read the box for more information about Jallikattu Law of 2017)

Religion - The religion practiced by Primitive communities played a decisive role in evolving the laws of the state later. Religion was a basis of law for most of the nation. The origin of Hindu law can be traced in the code of Manu. The origin of Mohammedian law can be traced in Sharia law. Divine law is a law revealed through man from God. God is the ultimate source of divine law. For Christians Ten Commandments were the first law given by the Lord Almighty to his people and was considered as the basis of law. “Indeed the early law of Rome was little more than a body of technical religious rules, a system of means for obtaining religious rights through the proper carrying out of certain religious formulas”. -*Woodrow Wilson*

Judicial decisions - Gettell maintains that the ‘state arose not as the creator of law but as the interpreter and enforcer of custom’. The function of the Judiciary is to interpret and declare the law. While discharging its function the judiciary creates new laws. The laws later gets recognized by the state. Judicial decisions thus became an another source of law. Some time the verdict of High Court and Supreme Court are treated as laws.

Equity - When laws are ambiguous and do not fit in, the principles of equity are applied and cases are decided according to common sense and fairness. “Equity is a body of rules existing

by the side of the original civil law, founded on distinct principles and claiming incidentally to supersede the civil law in virtue of a superior sanctity inherent in those principles.” - *Sir Henry Maine*

Scientific commentaries - Another source of law are scientific commentaries. when the commentary appears it is understood only as an argument, later on its authority is recognized as more authoritative than the Judicial decision.

Legislature - Most of the laws in the modern times are framed by legislature and it is one of the most important source of law. Indian constitution is a classical example where the best provisions of other constitutions are borrowed and made it available for the best of our nation “The state is founded on the minds of its citizens, who are moral agents.. a bad people means a bad state and a bad laws.”

Types Of Laws

Criminal Law

Health care in hospital institutions have progressed and improved quality, but it is still inseparable from the public spotlight. Even though, there are many laws and regulations that regulate health care and criminal law in medical field. In the Criminal Code regulated about several criminal law in the medical field. Then, to follow the development of the community, so special legislation in the health field was established, such as Law Number 36 Year 2009 on Health, Law Number 44 Year 2009 on Hospitals, Law Number 29 Year 2004 on Medical Practice. In Law Number 36 Year 2009 on Health in Article 2 it is stated that health development is carried out based on humanity, balance, benefits, protection, respect for rights and obligations, justice, gender and non-discrimination and religious norms. Criminal laws are used to punish a defendant on behalf of the state and the public and deter future criminal behavior. Because criminal charges do not compensate victims of preventable harms, those individuals may have to pursue an action in civil court for damages. The same criminal defendant can become a civil defendant simultaneously, and a state or federal court must apply completely different legal standards for finding an injury and compensating a plaintiff patient.

Tort law

Tort law seeks to compensate victims of certain actions or inactions based on the breach of a legal duty that caused damages. Plaintiff patients seeking redress for a preventable harm can file a tort, or personal injury, suit in state or federal court and must establish that the defendant's action met the standards for the criteria of duty, breach, causation, and damages. The defendant provider or facility then may present evidence to defend the suit and show that these four requirements were not met.

In tort law, providers and facilities connected to a preventable harm can be named in a suit as defendants based on their respective roles and duties; accordingly, employers may be vicariously liable for employees' actions, corporate entities can be held liable for their affiliated facilities, and providers can be jointly and severally liable for actions for which they each held responsibility (*Insko v. Aetna Health and Life Ins. Co.*, 2009). State courts use case law decisions that penalize defendants and compensate plaintiffs to uphold "private rights of action." These decisions could have consequences for future plaintiffs with the same complaints.

For healthcare providers and facilities sued based on the failure to meet a certain "standard of care," the legal standard of care does not equate directly to a medical standard of care. The legal standard simply asks that a provider follow the same course as a reasonable provider would under the same circumstances: there are no technical requirements as in a medical standard of care (Moffett and Moore, 2011). A jury that finds that the requirements for a tort suit are met will find the defendant liable for the plaintiff's damages. These damages may be considered "compensatory," reflecting the monetizable damages sustained, or "punitive," scaled to effectively penalize the defendant.

Laws of Contract

Contract law issues such as non-compete agreements, call schedules, tail insurance, compensation methods, and ownership of patient records must be carefully evaluated, understood and often negotiated and changed in order to create a work environment where the employees' and employers' needs are met. In addition, Stark, Anti-Kickback and other healthcare laws and regulations often relate to the contract relationship and should be addressed.

Issues that remain unclear or unresolved in employment contracts can often result in irreparable harm to relationships between the parties with either the healthcare employee or employer choosing to terminate or not to continue the relationship.

Labour Laws

The employee relations landscape in Kenya is informed by Article 41 of the Constitution of Kenya 2010, which grants every person the right to fair labour practices, including fair remuneration, reasonable working conditions, and the right to subscribe to and participate in trade union activities. Article 24(1) of the Constitution further sets out that the enjoyment of rights and fundamental freedoms by any person should not prejudice the rights and fundamental freedoms of others. Health service provision falls within the "essential services" category. Section 81 (1) of the Labour Relations Act, 2007, defines essential service as, "A service the interruption of which would endanger the life of a person or health of the population or any part of the population." Section 78(1) of the Labour Relations Act, 2007, states that no person should take part in a strike or lockout if the employee and employer were engaged in an essential service. Section 81(3) provides that there shall be no strike or lockout for an essential service, and Section 81(4) provides that any trade dispute in a service listed as essential shall be adjudicated upon by the Industrial Court. Since the promulgation of the Constitution in 2010, health workers in the public service have, on several occasions, participated in industrial

action. The escalation of unrest amongst health workers in many counties, which often causes disruption of health services, is to blame for needless loss of lives. The Ministry of Health is committed to stemming escalating unrest in the health sector, as well as cultivating cordial relations amongst health workers, unions, and the health sector leaders. The development of user guide on employee relations for the health sector in Kenya is a critical step towards meeting this commitment. The user guide aligns with the Labour Laws and the Constitution of Kenya 2010, and seek amicable resolution of health workers' concerns and disputes, providing opportunity for dialogue at all avenues.

Acts of Parliament related to health

Act No. 21 of 2017 - An Act of Parliament to establish a unified health system, to coordinate the inter-relationship between the national government and county government health systems, to provide for regulation of health care service and health care service providers, health products and health technologies and for connected purposes.

Public Health Officers (Training, Registration and Licensing) Act, 2013

An Act of Parliament to make provision for the training, registration and licensing of public health officers and public health technicians, to regulate their practice, to provide for the establishment, powers and functions of the Public Health Officers and Public Health Technicians Council and for connected purposes

Case study

Background: Occasionally, a patient in critical care can die no matter what we do. Others may have had a good chance of recovery if admitted, but then limited resources impact on admission. Profound ethical questions confront the medical personnel as they watch and wait helplessly. **Setting:** Critical Care Unit, Moi Teaching and Referral Hospital, Eldoret **Objectives:** To explore dilemmas on end of life which have a heavy bearing on Doctors and Nurses: legally, morally and ethically.

Materials and methods: A case study of ZX derived from the local setting involving a patient who was in the Critical Care Unit for one year sixty five days. This was experiential: a best case scenario. Consent to highlight and publish the case study was sought from a relative of the deceased. The case study consist of a content analysis of the patient file, ethical issues arising were noted and were cross-checked to ensure they reflect the entries in the patient file. This study seeks to dissect some ethical issues experienced in management of ZX.

Conclusion: Modern health care has given rise to extremely complex and multifaceted ethical dilemmas. More often the health care workers (Physicians/nurses) are unprepared to manage these competently. There is no Law or guidelines in Kenya on end-of-life issues, making ethical

dilemmas inevitable. ZX case brings out four topics that pose particularly vexing problems to the health care givers in their practice of medicine: Respect and equal treatment; Communication and consent; Decision making for incompetent patients; and end of life issues. Professional health care providers associations in Kenya should issue pragmatic policy guidelines to safeguard the Kenyan medics.

Learning Activities

Practical activities

- Identify the roles and responsibilities of perioperative theatre technologist.
- Identify medical- legal issues in PTT.
- Acquire Medical legal guidelines in PTT
- Determine the nature of the offence in PTT
- Identify appropriate enforcement officers for medical legal issues
- Communicate to the enforcement team about a medical legal issue
- Record Medical legal- ethical issues in PTT
- Prepare and disseminated Medical legal- ethical issues reports

Self-Assessment

You are provided with the following questions for self -assessment, attempt them and check your responses.

- i. What are the roles and responsibilities of perioperative theatre technologist?
- ii. Which are th medical- legal issues in PTT?
- iii. What are the Medical legal guidelines in PTT?
- iv. What are the nature of offences in PTT?
- v. Which are the appropriate enforcement officers for medical legal issues?
- vi. How do you communicate to the enforcement team about a medical legal issue?
- vii. How do you record Medical legal- ethical issues in PTT?
- viii. How do you prepare and disseminated Medical legal- ethical issues reports

Tools, Equipment, Supplies and Materials

.....Find out as per the set up.....

Learning Outcome 2: Apply medical legal ethics in PTT

Introduction to the learning outcome

This learning outcome specifies competencies in identifying medical legal offences and implications related to PTT which are applied as per the relevant regulations. The regulations are identifiable as per the legal system as stipulated in the laws relating to the organization, the Employment Act and the medical ethics.

Performance standards

- i. Medical legal offences related to PTT are identified and applied as per given relevant regulations/law and work place policy.
- ii. Medical -legal implications related to PTT are identified as per the work place policy and relevant source of law.
- iii. Medical legal regulations are implemented as per given legal system.

Information Sheet

Law and Morality

Law and morality are two normative systems that control and regulate behaviors in a human community so as to allow harmonious and effective intersubjectivity between individuals who recognize one another as bearers of rights. The two notions have their common foundation in the concept of individual autonomy and equal respect for everyone. They have a complementary relationship. Law compensates for the functional weaknesses of morality and morality tempers the mechanical implementation of positive law through the notions of solidarity and responsibility (Jerome E, 1989). Different legal thinkers have interpreted the relationship between law and morality in different ways. On the one hand, there are those who argue that law and morality are independent – though not unrelated. For this first group, a law cannot be disregarded merely because it is morally indefensible (Alexy,2002). On the other hand, there are those who maintain that law and morality are interdependent.

The relationship between law and morality can be stated in the following manner:

Law is related to morality in the setting forth of those virtues that are related to the common good. This does not mean that positive human law should prohibit all vices nor command all virtues: rather it prohibits only the grosser failings of mankind which threaten the very survival of society and commands those virtues which can be ordained by human means to the common good.

Law is related to morality by the moral obligation imposed, i.e., by the necessity of an act in relation to a necessary end-since law as the command of practical reason necessarily implies an obligation.

Law is related to morality inasmuch as law is subject to and cannot contradict moral principles, i.e., natural moral law.

Law is related to morality inasmuch as both stem and are directed by the same source: practical reason or prudence.

Medical-legal issues and ethics in health

The main legal issues in the healthcare system relate to medical negligence, informed consent and confidentiality. There are several ethical issues faced by health practitioners and patients in the healthcare sector

Confidentiality and Privacy of patients - The protection of private patient information is one of the most important ethical and legal issues in the field of healthcare. Conversations between a physician and a patient are strictly confidential, as is information about an individual's medical condition. Specific provisions of the Health Insurance Portability and Accountability Act, or HIPAA, define exactly what information can be released and to whom.

Healthcare system malpractice and tort reform – The main reason why governments work on reducing medical malpractice and have implemented tort reform is to reduce expenditure on unnecessary healthcare services. Besides reducing overall health costs, these reforms can also lead to a decrease in global malpractice risk.

Physician-assisted Suicide (PAS) and euthanasia – Euthanasia refers to the process of a deliberately ending a patient's life in order to relieve them of suffering. It is illegal in most countries but is still a hotly debated ethical issue surrounding whether it should or should not be legalized. PAS is the practice of providing a competent patient with a prescription for medication that will end their life. In this case, the patient would have to self-administer the medication, directly or through a machine.

Transmission of Diseases - The recent global outbreak of the Corona virus put a renewed emphasis on the right of healthcare providers to protect themselves from communicable diseases, whether by direct or indirect contact with an infected patient. Ethical and legal questions arise when a patient's health history is not provided to the medical staff.

Informed consent – Informed consent means providing an in-depth understanding of the risks and benefits of a treatment to a patient. This allows the patient to decide whether or not they wish to undergo a procedure. Informed consent is based on the principle that patients should have autonomy over the treatment they receive. Healthcare providers should have a document providing proof the patient has full knowledge of the risks, benefits and possible consequences of a particular treatment

Laws relating to persons and organizations

Considering all aspects of health, there are rules that regulate the rights and responsibilities of governments, health practitioners, organizations, civil society and a country's population. These rules make up the legal framework for health. They are classified in different forms including: statutory laws, regulatory and administrative laws, contracts, case law, and customary laws. WHO is mandated in making these rules as per every country's requirement.

Health laws are used to establish commitment to goals, and therefore creating a drive for action. To achieve health goals, law is required to create different organizations and contracts that provide health services. In turn, organizations such as health ministries, the private sector or civil society, have mandates, policies and strategies based on legal rules that guide their work.

There are also many rules that structure what health organizations and individuals should do, and what they may not do. This interaction between different health laws results in health system functions being carried out and health products and services being delivered.

Employment law and Employment Act

Some employee relations that relate to the health sector are stipulated as per the following:

Article 41 of the Constitution of Kenya 2010 - Grants every person the right to fair labour practices, including fair remuneration, reasonable working conditions, and the right to subscribe to and participate in trade union activities.

Article 24(1) of the Constitution - Sets out that the enjoyment of rights and fundamental freedoms by any person should not prejudice the rights and fundamental freedoms of others. Health service provision falls within the “essential services” category.

Section 81 (1) of the Labour Relations Act, 2007 - Defines essential service as, “A service the interruption of which would endanger the life of a person or health of the population or any part of the population.”

Section 78(1) of the Labour Relations Act, 2007 - States that no person should take part in a strike or lockout if the employee and employer were engaged in an essential service.

Section 81(3) of the Labour Relations Act, 2007 - Provides that there shall be no strike or lockout for an essential service.

Section 81(4) of the Labour Relations Act, 2007 - Provides that any trade dispute in a service listed as essential shall be adjudicated upon by the Industrial Court.

Health Act, 2017 - Established a unified health system, to coordinate the inter-relationship between the national government and county government health systems, to provide for regulation of health care service and health care service providers, health products and health technologies and for connected purposes

Medical Ethics

Medical ethics involves examining a specific problem, usually a clinical case, and using values, facts, and logic to decide what the best course of action should be.

Medical ethics are set on a foundation of some core principles which include:

Autonomy

Respecting patients privacy, confidentiality and being truthful about their medical care.

Provide the information and allow the patients to make their own decisions regarding their care .

Honor and respect the patients' decisions of either accepting or declining care or information.

Beneficence

Do everything good in regard to the patient

Nonmaleficence

Avoid causing injury or suffering to patients

Justice

Treat patients fairly and equitably.

Learning Outcome 3: Liaise with law enforcement team on medical legal and ethical issues

Introduction to the Learning Outcome

This learning outcome specifies competencies in identifying nature of the offences and the liable enforcement officers as per the laws and the workplace policies. Effective communication to the enforcement team is also taken as per the communication channels in the administrative structure.

Performance standard

- i. Nature of the offence is identified as per given regulations/laws and workplace policy.
- ii. Enforcement officers are identified according to work place policy and given regulations/laws.
- iii. Communication to the enforcement team is carried out as per communication channels and work place policy.

Information Sheet

Enforcement Team

National Police Service

The Article 243 of the constitution of Kenya established Kenya's National Police Service as the umbrella law enforcement organ in Kenya after a dissolution of Kenya Police Force and Administration Police Force. The NPS is under the overall and independent command of the Inspector-General, who is appointed by the President and approved by Parliament.

Medical Practitioners and Dentists Board

The medical Practitioners and dentist board is established by the constitution of Kenya with a mandate to regulate the practice of medicine, dentistry and medical institutions.

The board's core functions are as follows:

Training – approval of training institutions for medical and dental practitioners

Registration – Register eligible medical and dental practitioners

Licensing – issue annual license to private practice specialists

Education – Monitor university examinations for medical and dental students.

Inspection – Inspect and accredit institutions for medical and dental training

Organization of the Judiciary

The Judiciary is one of the three arms of government established under chapter 10, Article 159 of the Constitution of Kenya. It is established as an independent custodian of justice, with the primary role being to exercise judicial authority. The key functions include:

Administration of justice

Formulation and implementation of judicial policies

Compilation and dissemination of case law and other legal information for the effective administration of justice

The Court system has been decentralized with the Supreme Court and the Court of Appeal having their own Presidents and the High Court having a Principal Judge as heads of the respective institutions. The Supreme Court of Kenya is established under Article 163 of the Constitution of Kenya. It comprises of Seven judges: the Chief Justice, who is the president of the Court, the Deputy Chief Justice, who is the vice-president of the Supreme Court and five other judges.

The High Court consists of a number of judges to be prescribed by an Act of Parliament. The Court is organized and administered in the manner prescribed by an Act of Parliament. The Court has a Principal Judge, who is elected by the judges of the High Court from among themselves.

The Judiciary also consists of the Environment and Land Court as well as the Employment and Labour Relations Court which are courts of equal status as the High Court.

The subordinate courts consists of the Magistrates' Courts, Kadhis Courts, Court Martial, and any other court or local Tribunal established by an Act of Parliament.



Health Organization Structure/Protocol

The executive is the Cabinet Secretary in the ministry of health, under which is the Principal Secretary and then the Director of Medical Services. The Director of Medical Services coordinates and oversees the functioning of the six departments, whose regulations include:

The Department of Preventive and Promotive Health is further subdivided into six other sectors tasks with: National Public Health, Disease Surveillance and Epidemic response, non-communicable diseases, family health, environmental health, and national public health laboratory independent of each other.

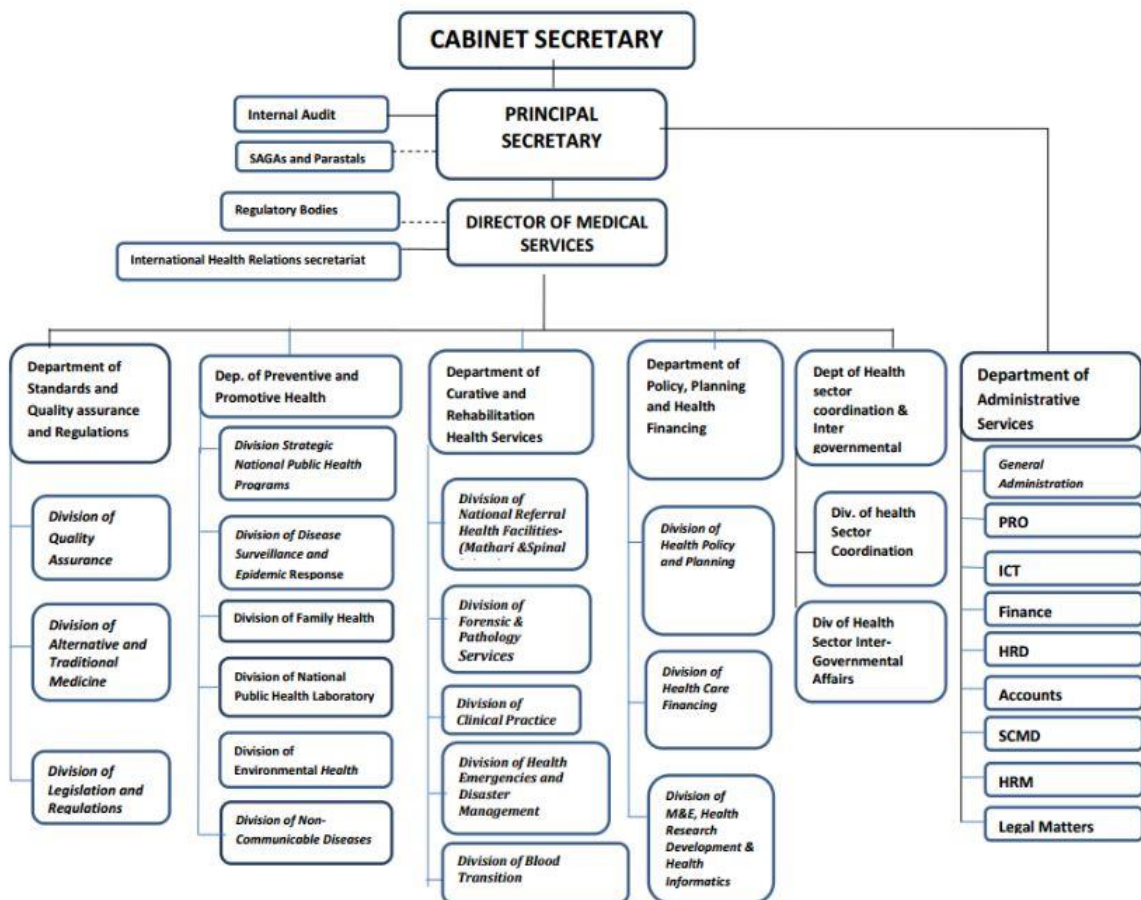
The Curative and Rehabilitation health services handle divisions of national referral services, forensic & pathology services, emergencies & disaster management, clinical practice and blood transition.

Standard and Quality assurance docket handles issues of alternative and traditional medicine, quality assurance, and legislation & regulation.

Health sector coordination & inter-governmental control department manages health sector intergovernmental affairs and coordination.

The administrative services functions include: monitors office operations such as finance, accounts, ICT, HRM, SCMD and legal matters.

Department of Policy, Planning and health financing which is the backbone of the Ministry of Health Kenya manages health policy & planning, Health care financing and the Health informatics, research development, and Monitoring & evaluation.



<https://www.health.go.ke/about-us/organizational-structure/>

Leadership and Interpersonal skills

Interpersonal skills help in understanding other people and adjusting your approach to work together effectively. For example, as a nurse you are required to comfort and care for the patients. interpersonal skills are therefore integral for the industry, especially empathy and patience.

Unlike technical or “hard” skills, interpersonal skills or “soft” skills are easily transferable across industries and positions. Employers value interpersonal skills because they contribute to positive work environments and help maintain an efficient workflow.

The following is a list of interpersonal skills:

Leadership - is an important interpersonal skill that involves effective decision making. Effective leaders incorporate many other interpersonal skills, like empathy and patience, to make decisions. Leadership skills can be used by both managers and individual contributors. In any role, employers value people who take ownership to reach common goals.

Active listening - means listening to others with the purpose of gathering information and engaging with the speaker. Active listeners avoid distracting behaviors while in conversation

with others. This can mean putting away or closing laptops or mobile devices while listening, and asking and answering questions when prompted.

Dependability – dependable people can be relied on in any given situation. This can include anything from being punctual to keeping promises. Employers highly value dependable workers and trust them with important tasks and duties.

Empathy - A worker's emotional intelligence is how well they understand the needs and feelings of others. Employers may hire empathetic or compassionate employees to create a positive, high-functioning workplace.

Teamwork - The ability to work together as a team is extremely valuable in every workplace. Teamwork involves many other interpersonal skills like communication, active listening, flexibility and responsibility. Those who are good team players are often given important tasks in the workplace and may be seen as good candidates for promotions.

Communication of Medical Legal Issues

Accurate and sensitive communication of health care information is key to effective patient management in all health care settings. Sensitive health care status of a patient may be embarrassing if it falls in the wrong hands. Medical ethics of medicine and statutory provisions emphasize that, the obligation of the physician is to safeguard patient confidences. However, threats on confidentiality of medical information are very important since communication has been characterized by growing use of email, facsimile, and the Internet.

Medical legal issues are in three key areas which include:

Confidentiality/breach of privacy - . Conversations between a physician and a patient are strictly confidential, as is information about an individual's medical condition (Keers-Sanchez A,2003).

Informed consent – provision of an in-depth understanding of the risks and benefits of a treatment to a patient is very important. This allows the patient to decide whether or not they wish to undergo a procedure. Informed consent is based on the principle that patients should have autonomy over the treatment they receive (Scioscia A,2000).

Defamation - defamation occurs when false words are spoken (slander) or written (libel) to a third party which exposes the individual to hatred, contempt, ridicule, or which causes the individual to be shunned or avoided, or which injures the individual or his or her business. Medical practitioners are especially sensitive to critical comments by patients. While hurtful, these comments are often opinions and are not necessarily considered defamatory because they do not contain facts that can be proven untrue (Harsham P,1984).

Administrative skills

Administrative professionals are required office-related tasks. For the most part, they are required to ensure that all clerical work and customer handling is performed to the highest standards of professionalism. The following skills are required for effective office administration:

Communication skills. - Office administrators are required to have proven written and oral communication skills. This is because, not only will they be the first point of contact to general public, but will also be required to carry out clerical tasks that involve exceptional levels of written communication. Spelling, grammar, and sentence structure should be polished to the highest standards when it comes to office administration.

Filing / paper management - Office administration staff will also need to show their ability to manage business correspondence. Filing, sorting, and general organization skills are essential for office administrators. In addition to this, administration staff will also need the ability to organize their own professional priorities.

Bookkeeping. - General office procedures are necessary for administrative staff, but many positions will require administration personnel to be familiar with bookkeeping. It is rare that this is done by hand nowadays, but there are several different computer applications that can be used to manage financial transactions, including Quicken, Excel, and Turbo Tax.

Typing. - Since ICT is essential for administrative staff in today's era, so too is the ability to type. Many positions don't denote a specific speed, but general opinion believes this to be around 50 words per minute (wpm) with 100% accuracy. There is no prescribed method of typing, but those who need to increase their speed may find touch-typing through the "home row" method to be appropriate for their needs, as it is the most widely taught method of typing and is available in different software tutorials.

Equipment handling. - Administrative staff may be called upon to do some light to moderate lifting, so should not see this as an opportunity to avoid laborious tasks. Fortunately, this is rather limited to tasks such as bringing reams of paper to a copy machine or printer.

Customer service skills. - Having excellent customer service and an orientation towards such traits is necessary for success as an administration worker. As candidates will be customer facing many times, they will need to carry themselves with both professionalism and have a pleasant demeanor.

Research skills. - Administration workers will need to have special talents in research and data analysis in order to compile reports and harvest research material for other workers within their department.

Self-motivation. - Administration personnel rarely have management working directly above them and monitoring their every move. However, any loss of drive or motivation will be immediately visible to them in terms of the quality of the work being received.

Flexibility. - Administration rarely has two similar days. While personnel can be sure of working a relatively static schedule within typical business hours, there may be occasions where working outside of the conventional schedule will be required.

Learning Outcome 4: Document medical legal- ethical issues in POTT

Introduction to the Learning Outcome

This learning outcome identifies competencies in proper recording, reporting and disseminating of medical legal-ethical issues to relevant authorities as per work place policy. Appropriate disposal of waste is key area in medical set up.

Performance standards

- i. Medical legal- ethical issues are recorded according to work place policy.
- ii. Medical legal- ethical issues reports are prepared and disseminated to relevant authorities as per work place policy.

Information Sheet

Documentation of medical legal and ethical issues

Clear medical record documentation is the key to providing patients with quality care, ensuring accurate and timely payment for the services furnished, mitigating malpractice risks, and helping healthcare providers evaluate and plan the patient's treatment and maintain the continuum of care (Gutheil TG,1980). Reasons for proper documentation of patients' medical records include:

Communicates with other health care personnel - Documentation communicates the quality of clinical care that providers are delivering to patients and serves as a means to facilitate the patient navigation continuum of care, from EM to HM and beyond (Gutheil TG,1997).

Reduces risk management exposure - Documentation mitigates risks and reduces the chance of malpractice. A well-documented record can help alleviate liability concerns in the event of a claim.

Ensures appropriate reimbursement - A well-documented medical record can facilitate effective revenue cycle processes, expedite payment, reduce any "hassles" associated with claims processing, and ensure appropriate reimbursement.

Types of medical records

Patient information sheet - you can find in any medicine bought at a pharmacy. It contains very important information about the product such as its conditions, side effects, doses or

conservation. A medical and healthcare translator must pay a special attention to the country regulations and quantities.

Medical history record - A medical history record summarizes a patient's medical history and also includes some notes and remarks made by healthcare specialists. It can be used in a clinical environment, and the medical and healthcare translator should never misunderstand the contained information.

Discharge Summary - A hospital discharge summary is prepared when a patient checks out of the hospital. It normally explains the reasons for the hospital admission, the results of the tests made, and how the patient feels after the discharge – with some medical advices.

Medical test - It includes a blood test, an urine test, a heart test, and many other variants. Figures, chemical forms, molecules, quantities... everything have to be carefully looked by the medical and healthcare translator.

Mental Status Examination - It details the results of a mental exam. Is the patient in his/her right mental abilities? A medical specialist will present the results in an official document which may required a medical translator.

Operative Report - It is a report prepared to detail the procedures and findings of a surgical operation. The medical and healthcare translator should avoid any ambiguity and be as clear and concise as possible.

Report writing and dissemination on medical legal issues

Structured reports support readers to interpret and understand the content of the report and may reduce subsequent requests for information. A request to provide a medical report will often be accompanied by a set of questions and/or a template or form. There is generally an accepted structure for medical reports that can be used when a template or form has not been supplied by the third party, or when a set of questions has been provided with no instructions. (Seidl I,2011) The recommended structure below may assist in preparing a comprehensive medical report (Australian Medical Association. GST, 2014).

Identify yourself and your credentials - The report should state who has prepared its contents and their scope of expertise. You can achieve this by providing a paragraph overview outlining your experience and expertise.

Introduce the scope and purpose of the report - The reason for preparation of the report should be clearly stated, including any questions you have been asked to consider/address.

Outline relevant history, outcomes of physical examination and management strategies - The report should contain a detailed medical history of events or illnesses that relate to the purpose for preparing the report. Information on the general state of health or other relevant factors should also be included, along with the proposed management of the condition.

Provide an opinion and/or respond to questions - This section should include responses to any specific questions provided by the third party. The process for reaching stated conclusions

should be transparent and substantiated within the report. Providing an outline of the clinical method employed and the reasoning/justification for your conclusions is recommended.

Principles for Disseminating Research Results

Medical researchers should assess ramifications of their research in an effort to balance the promise of benefit against potential harms from corrupt application. Only under rare circumstances should findings be withheld, and then only to the extent required to reasonably protect against misuse (World Medical Association, 2013). To fulfill their ethical responsibilities with respect to sharing research findings for the ultimate benefit of patients, physicians should:

Advocate for timely and transparent dissemination of research data and findings. Physicians should not intentionally withhold information for reasons of personal gain.

Report the results of research accurately, including subsequent negative findings. This is particularly important where the findings do not support the research hypothesis.

Maintain a commitment to peer review.

Disclose sponsorship and conflicts of interest relating to the research, in keeping with ethics guidance.

Be responsible in their release of research results to the media, ensuring that any information the researcher provides is prompt and accurate and that informed consent to the release of information has been obtained from research participants (or participants' legally authorized representative when the participant lacks decision-making capacity) prior to releasing any identifiable information (Schroter S, 2014).

Waste Disposal

Sharps disposal is one of the most important duties of any healthcare worker. Proper disposal keeps us safe from bloodborne pathogens. *Improper* disposal opens us up to infection and hefty fines. The key to staying safe from needle sticks and cuts is knowing what sharps are, *and* how to dispose of them. A reliable disposal company can carry the lion's share of the sharps disposal burden.

Types of medical waste:

Infectious waste: waste contaminated with blood / blood by-products, cultures and stocks of infectious agents, waste originating from isolation ward patients, discarded diagnostic samples containing blood and body fluids, infected animals originating from laboratories, and contaminated materials

- i. Pathological waste: identifiable human body parts and contaminated animal carcasses.
- ii. Sharps: disposable scalpels and blades, syringes, needles, etc.
- iii. Chemicals: mercury, solvents, disinfectants etc.

- iv. Pharmaceuticals: unused, contaminated, and out-of-date medicines; vaccines and serums.
- v. Genotoxic waste: very hazardous teratogenic, mutagenic, or carcinogenic waste, such as cytotoxic medicines for the treatment of cancer and their metabolites.
- vi. Radioactive waste: for example glassware contaminated with radioactive diagnostic matter or materials used during radiotherapy.
- vii. Heavy metal waste: for example mercury thermometers.

Methods of waste disposal

Incineration - This is the process of burning of waste in temperatures ranging from 1,800°F to 2,000°F (982°C to 1093°C). On-site incineration provides the advantage of a quick, easy disposal method, but there are emissions concerns.

Autoclaving - Autoclaving, which is also known simply as steam sterilization, is the most commonly utilized alternative to incineration. It is both less costly and carries no documented health impacts. In this method, wastes are sterilized or disinfected prior to disposal in a landfill. Bags of waste are placed in a chamber and steam is introduced for a determined period of time at a specified pressure and temperature. This assures the destruction of microorganisms(Pépin J,2014).

Gas sterilization - In this process, medical waste is fed into an evacuated air-tight chamber and treated with a sterilizing agent (such as ethylene oxide or formaldehyde). The gas that comes into contact with the waste will kill harmful, infectious agents.

Chemical disinfection - This process involves the use of chemical agents for disinfection, such as chlorine. Chemical disinfection processes are most appropriate for liquid wastes, although they can still be used to treat solid wastes. There are a number of factors that should be considered regarding the effective usage, including: the types and biology of microorganisms in the wastes, degree of contamination, type of disinfectant used as well as its concentration and quantity, contact time, and mixing requirements.

Microwave - The application of microwave technology treatment also can disinfect waste. Waste is first shredded, and then mixed with water and internally heated to neutralize all present biologicals. Computerized controls are employed to ensure the minimum parameters for disinfection and proper equipment function. As with autoclaving, approximately 90% of medical wastes can be treated with this process. The shredding process results in a volume reduction, and energy use is reportedly lower than that of an incinerator.

Irradiation - This method involves sterilizing waste by exposing it to a cobalt source. Cobalt gives out gamma radiations that destroy all microbes in waste. The high cost of cobalt and high operating costs, however, have discouraged commercial ventures from using the technology for the treatment and management of medical waste. There also have been some questions raised about the actual process of radiating the material and achieving adequate disinfection. As with autoclaving and microwave methods, it is not recommended for pathological wastes(WHO/UNICEF,2015).

Thermal inactivation - This process involves heating waste to temperatures at which infectious agents are killed. It is used for treating large volumes of liquid clinical wastes. A chamber is preheated to an intense, specified temperature and held for a specified time, then released.